

MANAGEMENT OF PSORIASIS

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Introduction

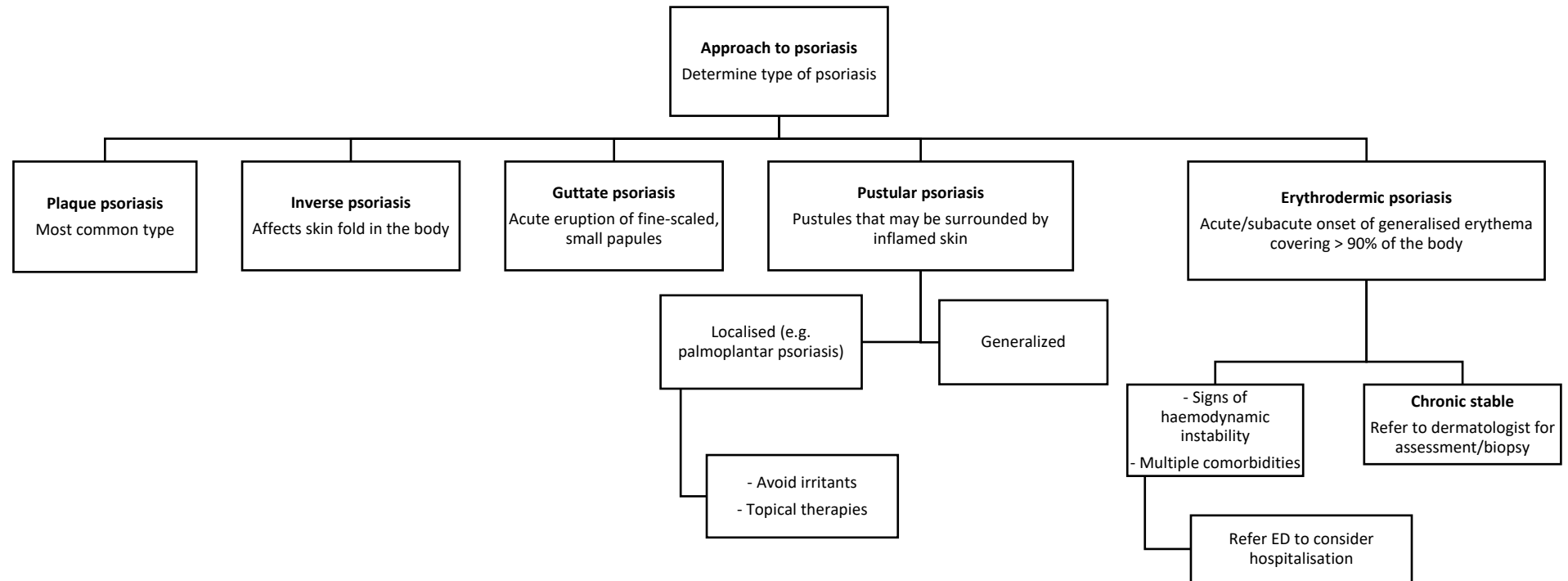
Psoriasis is a chronic, relapsing and remitting, non-infectious inflammatory skin disease characterised by well-demarcated thick scaling erythematous plaques. It has a bimodal age of onset (16-22 years and 57-60 years) and affects both sexes equally. Psoriasis is an immune-mediated disease with genetic predisposition.

Distribution:

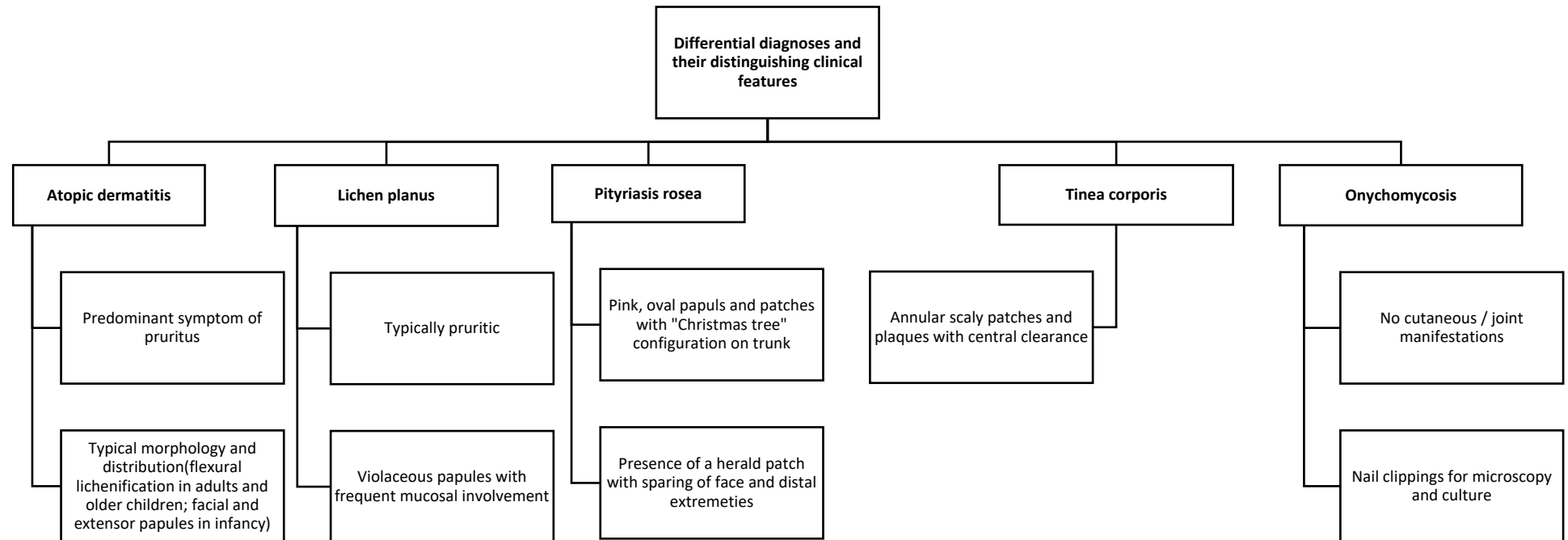
Usually the extensor surfaces (elbows, knees), scalp, nails.

Intertriginous areas (e.g., axilla, groin folds, natal cleft) can sometimes be involved.

Approach to Psoriasis



Differential Diagnoses



Management of Psoriasis

1. Non-Pharmacological

- Assess impact of disease on patient and psychological distress
- Identify and avoid triggers
- Consider replacing potentially inducing or aggravating drugs, where clinically appropriate (e.g., beta-blockers)
- Avoid scratching / trauma
- Counsel on support groups (Psoriasis Association of Singapore)
- Advise on weight management, reduce alcohol intake and smoking cessation to reduce cardiovascular risk factors
- Screen and manage comorbidities (e.g., lipid panel and fasting glucose) as psoriasis has association with metabolic syndrome
- Look for nail or joint involvement

2. Stable Chronic Plaque Psoriasis

- First line topical therapy:
 - Betamethasone 0.1% cream BD or Mometasone 0.1% cream OD for plaques on trunk and limbs, with aim to taper to betamethasone 0.05% cream BD when better.
Betamethasone 0.025% cream BD for plaques on face, and flexures
 - If scalp is involved, consider betamethasone 0.1% scalp lotion BD. Coal tar shampoo or cetrimide shampoo twice weekly or more often if needed.
 - Coal tar 10% in aqueous cream OD/BD can be used as emollient substitute.
** Tar products may potentially stain skin, hair, or clothing. Patients may also find the odor unpleasant.*
- Second line topical therapy:
 - Ointment equivalent topicals can be used for thicker plaques (e.g. betamethasone 0.1% ointment BD for thicker plaques on trunk and limbs or betamethasone 0.025% ointment BD for thicker plaques on face and flexures).
 - Betamethasone dipropionate 0.05% and salicylic acid 3% ointment (Betacyclic) BD: Use limited to resistant lesions. Once the lesions have flattened, it is recommended to switch to a topical corticosteroid of lower strength until complete resolution.

** Other treatments not available in polyclinic include: Topical vitamin D3 analogue (Calcipotriol), phototherapy, systemic therapy e.g., acitretin, methotrexate, cyclosporine, and biologics.*

3. Specialist Consultation

- Unsatisfactory response to topical treatments such as steroids or coal tar
- Psoriasis affecting 3-10% BSA (a person's palm is approximately 1% BSA)
- Patients who may benefit from phototherapy or systemic therapy
- Pustular / Erythrodermic Psoriasis
- Psoriatic Arthropathy
- Uncertain diagnosis

References

1. Guideline for Escalation of Therapy in the Primary Care Setting for Common Skin Conditions (Document shared by Dermatologist from National Skin Centre in April 2024)
2. WL Lim, CH How, KW Tan. Management of psoriasis in primary care. Singapore Med J 2021; 62(3): 109-112